INFORMATION TO KEEP WITH YOU

# My complete treatment handbook

## Dose-dense AC → weekly paclitaxel

Understand your treatment. Know what to expect. Keep your plan to hand.

**Displayed calendar**
**14 days**
Intervals differ by treatment phase

**Treatment stages**
**2**
Explained in the treatment reference

**Regimen source**
**NCCP 00485**
Version 6

Four AC treatments every 14 days, then 12 weekly paclitaxel doses starting 14 days after the last AC. About 20 weeks without delays.

## Treatment stages

01 **AC chemotherapy**
4 cycles, every 14 days

02 **Weekly paclitaxel**
12 weekly doses

Your team confirms the active stage and dates. The full treatment guide explains each step.

## Inside your handbook


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**My name** _____

**Hospital / oncology team** _____

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

# Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

## When to get help

**Emergency: call 112 / 999**

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

**Contact your oncology service immediately**

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

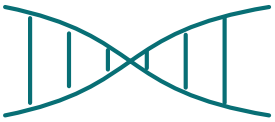
This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

## Your medicines and what they do

Reduce recurrence risk as part of the agreed early-breast treatment plan.

ANTHRACYCLINE CHEMOTHERAPY

### Doxorubicin



**Interferes with deoxyribonucleic acid (the cell's genetic material) (DNA) handling**

Doxorubicin interferes with enzymes and DNA processes needed for cell division. It can also affect heart muscle, so the team considers previous exposure and cardiac risk.

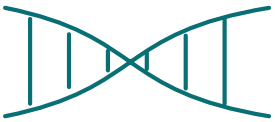
DNA handling disrupted → Cell damage → Division reduced

**What to watch for**

Report pain or burning at the infusion site immediately. New breathlessness or chest symptoms need prompt review. Red urine for a short time can be a drug-colour effect; pain, clots or persistent bleeding still need advice.

ALKYLATING CHEMOTHERAPY

## Cyclophosphamide



### Damages DNA needed for growth

Cyclophosphamide is activated in the body and damages DNA. This helps stop dividing cancer cells but can also affect blood-cell production and reproductive tissues.

Drug activated → DNA damage → Cell division affected

### What to watch for

Use the prescribed anti-sickness and hydration plan. Report fever, painful urination or blood in the urine.

TAXANE CHEMOTHERAPY

### Paclitaxel

Taxol (conventional paclitaxel)



**Locks the cell division machinery**

Cells use tiny structures called microtubules to separate their chromosomes when dividing. Paclitaxel stabilises these structures so they cannot rearrange normally, interrupting cell division. It can also affect nerves and other healthy tissues.

Microtubules form → Structures remain locked → Division is interrupted

**What to watch for**

Report tingling or numbness before the next weekly dose, especially if it affects walking or hand use. Tell the nurse immediately about flushing, chest or throat tightness, itching, dizziness or breathing changes during infusion.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

## Why this treatment?

This anthracycline–taxane sequence is an option for selected high-risk early breast cancer. Dose-dense AC needs growth-factor support; the later weekly paclitaxel phase has different premedicines and toxicity priorities.

**Your treatment setting**

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer

**Your exact schedule**

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks.

**Alternatives**

Discuss biomarker-directed, endocrine, chemotherapy and supportive-care options appropriate to this setting, including the option not to proceed.

## Why this treatment may be chosen

Discuss how this evidence and the available alternatives apply to your own situation.

CHAPTER 02

# Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

# Treatment and review

### Treatment stages

01 **AC chemotherapy**
4 cycles, every 14 days

02 **Weekly paclitaxel**
12 weekly doses

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

4 CYCLES, EVERY 14 DAYS
### AC chemotherapy

**4 cycles, every 14 days**

Growth-factor support is required for each dose-dense AC cycle. The anti-sickness prescription belongs to this phase.

12 WEEKLY DOSES
### Weekly paclitaxel

**12 weekly doses**

Premedicines and anti-sickness support change. Bloods and numbness are checked before each weekly dose.

Your team confirms the active medicines, treatment days, dose changes and next review.

BASELINE → TREATMENT → FOLLOW-UP

## When checks happen

Relative milestones, not equal time intervals. Your team books tests and reviews results; this map does not confirm completion.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00485 v6 · official regimen · ESMO 2024 early breast cancer guideline · treatment and follow-up · NICE NG101 · bone health and follow-up

THE WHOLE TREATMENT JOURNEY

## Dose-dense AC → weekly paclitaxel: treatment and checks

**Read from left to right.** Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

**Treatment, tests and reviews across the course**

| Care / check | Before starting | First treatment | During treatment | Change / completion | Follow-up |
|---|---|---|---|---|---|
| **Treatment plan**<br>NCCP 00485 | Eligibility and cardiac risk | AC q14 days ×4 | Paclitaxel weekly ×12 | Agree local/ endocrine treatment | Early-breast follow-up |
|  <br>**Blood / organ function**<br>NCCP 00485 | full blood count (FBC), renal, liver baseline | Review results before AC | FBC, kidney and liver tests before each cycle; team confirms checks before weekly paclitaxel doses | Confirm recovery before restart / transition | Investigate persistent problems |
| **Heart / nerves**<br>NCCP 00485 |  <br>echocardiogram (heart ultrasound) (ECHO)/ multigated acquisition (heart pumping) scan (MUGA) if >65 or indicated; doxorubicin left ventricular ejection fraction (heart pumping measurement) (LVEF) >50% | Review symptoms / anthracycline exposure | Cardiac review as indicated; taxane neuropathy | Review cumulative dose and symptoms | Report new cardiac symptoms |
| **Support changes**<br>NCCP 00485 | Agree growth-factor product / days | granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) required during dose-dense AC | Switch to weekly-paclitaxel premedicines | Do not carry AC support across automatically | Review continuing support needs |
|  <br>**Cancer review**<br>EARLY | Confirm setting | Agree review plan | Follow the early-breast plan | Next treatment / completion | Agreed surveillance |

## What each check is for

### 1 Before treatment

Before the first dose

Adjuvant or neoadjuvant high-risk node-negative or node-positive breast cancer. FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

**Ask / act:** Confirm eligibility, test results and medicine list.

### 2 Other checks

At the specified review points

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

**Ask / act:** Confirm who arranges and reviews each check.

### 3 Support and practical plan

At each treatment or phase change

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

**Ask / act:** Choose the matching support plan; confirm active medicines.

### 4 Response and follow-up

Agreed review points

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine computed tomography (CT)/positron emission tomography (PET) scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns. Keep the agreed follow-up and report persistent or new symptoms between visits.

**Ask / act:** Review symptoms, results and patient priorities.

## At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

## Your treatment and access plan

STEP 1

### Before treatment

FBC, kidney and liver tests at baseline and before each cycle, with the unit confirming weekly paclitaxel test requirements. Review current illness and past dose delays separately.

STEP 2

### Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.



STEP 3

### On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

### After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

#### Visit length

AC and weekly paclitaxel have different visit and supportive-medicine plans. The unit confirms the complete appointment duration.

#### Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

#### Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

#### Support for this visit

G-CSF is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan. First paclitaxel dose: dexamethasone 8 mg IV, chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).

## Checks across treatment

Your team arranges tests and reviews the results before treatment decisions.

### Blood and organ checks

### Other treatment-specific checks

Review lifetime anthracycline exposure and cardiac risk. Echo/MUGA is indicated above age 65 or when clinically indicated; the source requires LVEF above 50% for doxorubicin. Reassess cardiac symptoms and neuropathy as indicated; no trastuzumab is included.

### Cancer review and follow-up

After early-breast treatment, breast imaging and clinical review follow the agreed plan. Routine CT/PET scans or blood tumour markers are not used to search for distant recurrence in someone without symptoms or clinical concerns.

### Bone health

Bone-health assessment follows endocrine therapy, ovarian suppression, menopause and other fracture risks. It has its own monitoring plan.

## The wider treatment plan

For neoadjuvant treatment, agree response review and surgery planning. After surgery, confirm radiotherapy and endocrine treatment where appropriate; their schedules and checks remain separate.

### Medicines review

Check new prescriptions, over-the-counter medicines and supplements with the oncology pharmacist before taking them.

### After treatment

Keep the agreed follow-up and report persistent or new symptoms between visits.

CHAPTER 03

# Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

# Phase 1 · AC every 14 days

## Day 1 → Day 14: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Four AC treatments 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel doses, subject to recovery and review.

| Treatment or check | Timing in this view |
|---|---|
| **Planned treatment** |  **Day 1** Treatment |
| **Nausea / appetite** |  **Days 1–5** Can occur |
| **Infection / low counts** |  **Days 1–14** Possible at any point |
| **Fatigue** |  **Days 1–14** May build over treatment |

**Timing varies:** Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

**Treatment-day changes:** A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

> **There are no guaranteed "safe days".** Fever, shivers, sudden illness, new breathing problems or severe symptoms need prompt advice on any day, including a treatment break.

# Phase 2 · weekly paclitaxel

## Day 1 → Day 7: a general pattern

Paclitaxel is repeated weekly for 12 doses.

| Treatment or check | Timing in this view |
|---|---|
| **Paclitaxel** |  **Day 1** IV |
| **Aches / fatigue** |  **Days 1–7** Timing varies |
| **Numbness / tingling** |  **Days 1–7** Can accumulate |
| **Bloods before next dose** |  **Day 7** Review |

**Timing varies:** Symptoms can occur on any day, including days without treatment. The bands are orientation, not predicted symptom-free periods.

**Treatment-day changes:** A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

> **Why these bands stay broad:** Planned treatment days are distinct from broad possible symptom patterns. Symptoms can occur outside these bands. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00485 v6 · official regimen ↗

### Between-treatment notes

### Day 1–14: Reference cycle

Four AC treatments (doxorubicin 60 mg/m² and cyclophosphamide 600 mg/m²) 14 days apart, followed 14 days after the last AC by 12 weekly paclitaxel 80 mg/m² doses, subject to recovery. The planned sequence spans about 20 weeks. Follow your written appointment and medicine plan. A calendar does not authorise a dose.

## Support around the medicines

**1** **Blood counts and infection**

Fever, shivers or sudden illness needs prompt advice on any day.

**2** **Nerve symptoms**

Report numbness, tingling or weakness before the next dose.

### 3 Support medicines

Follow the prescription for the active medicine and treatment day.

### 4 Review the plan

Discuss symptoms, recovery and the next treatment decision with the team.

## Questions worth asking

**Who arranges the checks and explains the results?**

Agree the due window, booking team, result reviewer and contact route if a test or appointment is missing. This page does not record completed checks.

**Try explaining the plan back:** Which medicines and doses are prescribed, when are they due, which checks are needed, and what would make you call the team today?

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**Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

## Phase 1 · dose-dense AC

**Active medicines:** Doxorubicin + Cyclophosphamide

**Emetogenic risk:** High · AC

| Medicine | Dose / route | When |
|---|---|---|
| Aprepitant | 125 mg orally | Day 1 |
| Ondansetron | 16 mg orally | Day 1<br>Individual dose / QT / hepatic review |
| Dexamethasone | 12 mg orally | Day 1<br>No routine Days 2–4 dexamethasone in this NCCP AC antiemetic row |
| Olanzapine | 5 mg orally | Days 1–4<br>Review sedation, falls, interactions and glucose |
| Aprepitant | 80 mg orally daily | Days 2–3 |
| Metoclopramide | 10 mg orally up to three times daily | Subsequent days: duration and use to be confirmed<br>Caution with olanzapine: additive extrapyramidal effects; adjust / avoid for individual contraindications |

### Additional phase-specific support

- granulocyte colony-stimulating factor (white-cell growth support) (G-CSF) is required for every dose-dense AC cycle. Prescribe product, dose and timing separately under the approved local plan.
- This script applies only to AC. It is replaced when weekly paclitaxel starts.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

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## Phase 2 · weekly paclitaxel

**Active medicines:** Paclitaxel

**Emetogenic risk:** Low

| Medicine | Dose / route | When |
|---|---|---|
| Ondansetron | 8 mg orally | On treatment day<br>Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed |
| Metoclopramide | 10 mg orally up to three times daily as needed | Subsequent days: duration to be confirmed<br>Assess contraindications, renal function and interactions |

### Additional phase-specific support

- First paclitaxel dose: dexamethasone 8 mg intravenous (IV), chlorphenamine 10 mg IV and famotidine 20 mg IV, each 30 minutes before treatment (NCCP suggested table).
- From Day 8 onward: chlorphenamine 10 mg IV and famotidine 20 mg IV 30 minutes before treatment. Routine dexamethasone may be omitted after the first dose; prior reaction or increased risk requires consultant review.
- Famotidine may be omitted under the consultant/local plan if appropriate. A prior reaction requires a separate rechallenge assessment; do not reuse the standard draft automatically.
- Growth-factor support is required during dose-dense AC; do not automatically continue it during weekly paclitaxel without clinical need.

**Before signing:** verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

# What to watch for

You do not need to know which medicine caused a symptom before calling.

### 1 Hair / scalp

**Chemotherapy**

Hair loss or thinning varies; ask about support.

### 2 Mouth

**Dose-dense AC → weekly paclitaxel**

Painful ulcers or difficulty drinking need advice.

### 3 Breathing

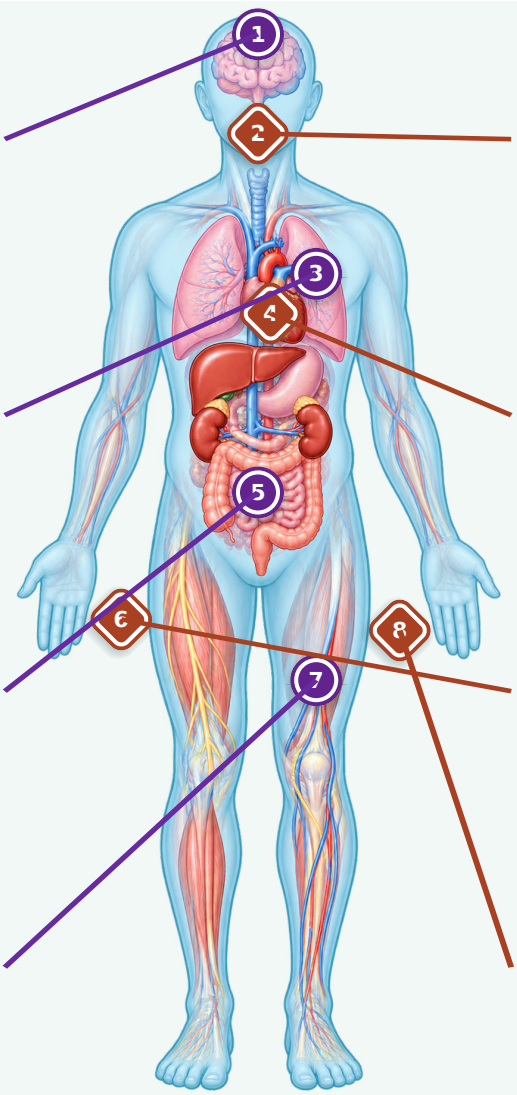
**Dose-dense AC → weekly paclitaxel**

New cough or breathlessness needs prompt assessment.

### 4 Heart

**Dose-dense AC → weekly paclitaxel**

New chest pain, fainting or severe breathlessness needs urgent assessment.




### 5 Bowel

**Dose-dense AC → weekly paclitaxel**

Report diarrhoea early, especially with poor intake or dizziness.

### 6 Nerves / joints

**Dose-dense AC → weekly paclitaxel**

Report numbness, tingling or pain affecting daily activity.

### 7 Blood / infection

**Dose-dense AC → weekly paclitaxel**

Fever, shivers, unusual bleeding or sudden illness: call promptly.

### 8 Skin / liver symptoms

**Dose-dense AC → weekly paclitaxel**

Report severe rash, blistering or yellow skin / eyes promptly.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Report new breathlessness, jaundice, marked bleeding, severe diarrhoea, inability to drink, chest symptoms or a severe rash promptly.

**When to get help**

**Call 112 / 999** for severe breathing difficulty, severe chest pain, collapse or a seizure.

**Call oncology immediately:** temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

**Oncology daytime:** 087 12345678 · **Out of hours:** 087 87654321

**EXAMPLE ONLY — team must replace both numbers before patient use.**

CHAPTER 04

# Sources and further information

## IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.


1. **National Cancer Control Programme (NCCP) 00485 v6 · official regimen**
   https://healthservice.hse.ie/documents/6500/485_v6_Dose_DenseAC-T.pdf
2. **ESMO 2024 early breast cancer guideline · treatment and follow-up**
   https://www.annalsofoncology.org/article/S0923-7534(23)05104-9/fulltext
3. **NICE NG101 · bone health and follow-up**
   https://www.nice.org.uk/guidance/ng101/chapter/Recommendations
4. **eviQ · implanted port patient information**
   https://www.eviq.org.au/p/507
5. **eviQ · peripherally inserted central catheter (PICC) patient information**
   https://www.eviq.org.au/p/543
6. **ESC 2022 cardio-oncology guideline · cardiac surveillance and risk**
   https://academic.oup.com/eurheartj/article/43/41/4229/6673995
7. **HSE · chemotherapy side effects and urgent advice**
   https://www2.hse.ie/conditions/chemotherapy/side-effects/
8. **NOGG · section 7, aromatase inhibitors and fracture risk**
   https://www.nogg.org.uk/full-guideline/section-7-strategies-management-osteoporosis-and-fracture-risk
9. **NCCP NCIS antiemetics v8 (2025), Table 1**
   https://healthservice.hse.ie/documents/7180/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
10. **ASCO · biomarkers in metastatic breast cancer**
    https://ascopubs.org/doi/10.1200/JCO.2015.61.1459
11. **ESMO metastatic breast cancer guideline · 2026**
    https://www.annalsofoncology.org/article/S0923-7534(26)00902-6/fulltext